

1. Administrative & Study Identification

Full Study Title: A Multicentre, Phase II, Single-Arm, Interventional Study of Neoadjuvant Durvalumab and Platinum-based Chemotherapy (CT), Followed by Either Surgery and Adjuvant Durvalumab or Chemoradiotherapy (CRT) and Consolidation Durvalumab, in Participants with Resectable or Borderline Resectable Stage IIB-IIIB Non-small Cell Lung Cancer (NSCLC)
Brief Title: Study to Assess Neoadjuvant Durvalumab (D) and Platinum-Based Chemotherapy (CT), Followed by Either Surgery and Adjuvant D or CRT and Consolidation D, in Resectable or Borderline Resectable Stage IIB-IIIB NSCLC (MDT-BRIDGE) **Short Title/Acronym:** MDT-BRIDGE **Protocol Number:** D9106C00002 **NCT Number:** NCT05925530 **Trial Status:** Active, Not Recruiting **Sponsor Name:** AstraZeneca **Investigational Product:** Durvalumab (MEDI4736; Trade Name: Imfinzi; ATC Code: L01FF03) in combination with Platinum-based Chemotherapy (CT) **Phase of Development:** Phase II **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the resection rate in all enrolled patients receiving neoadjuvant durvalumab and platinum-based chemotherapy followed by MDT-guided surgery or definitive chemoradiotherapy. **Secondary Objectives:** To evaluate resection rates by baseline resectable/borderline-resectable status, R0, R1, R2 resection rates, surgical resection outcomes, Event-Free Survival (EFS), Progression-Free Survival (PFS), Overall Survival (OS), Pathological Complete Response (pCR) rate, circulating tumor DNA (ctDNA) dynamics pre- and post-surgery, and safety.

2.2 Background & Rationale In the AEGEAN trial, neoadjuvant durvalumab plus platinum-based chemotherapy (D+CT) followed by adjuvant durvalumab significantly improved the pathological complete response (pCR) rate and event-free survival (EFS) in patients with resectable NSCLC. Strong clinical outcomes with chemoimmunotherapy have generated significant interest in its use to enable patients with borderline-resectable NSCLC to undergo successful surgery. Furthermore, for patients who initially present as borderline but become unresectable/inoperable during neoadjuvant treatment, consolidation immunotherapy following chemoradiotherapy (CRT) provides an important, evidence-based alternative pathway.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional Control Type: Single-arm Blinding: Open-label (None) Randomization: N/A (Single Group Assignment)

3.2 Planned Enrollment & Duration Target N\$: 142 adult patients Number of Sites: Multicenter, Global Estimated Study Start: 22-Feb-2024 Estimated Primary Completion: 12-Jan-2026

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Male or female participants ≥ 18 years at the time of screening. 2. Histologically or cytologically documented NSCLC. Previously untreated and pathologically confirmed Stage IIB to select (i.e., N2) Stage IIIB by AJCC v8. Nodal status must be confirmed with whole-body FDG-PET and biopsy (via endobronchial ultrasound, mediastinoscopy, or thoracoscopy), with a mandatory brain MRI. 3. Deemed resectable or borderline resectable at baseline, confirmed by Multidisciplinary Team (MDT) evaluation at diagnosis. 4. EGFR and ALK wild-type. 5. Medically operable: adequate cardiac and lung function to undergo resection, minimum life expectancy of 12 weeks, and minimum body weight of 30 kg. 6. Male and female participants must be willing to use acceptable methods of contraception.

4.2 Exclusion Criteria 1. Female participants who are pregnant or breastfeeding (negative pregnancy test required for women of childbearing potential). 2. Tumors harboring EGFR mutations or ALK translocations (known EGFR mutation or ALK translocation). 3. Unacceptable toxicity or inability to tolerate platinum-based chemotherapy. 4. Patients deemed definitively unresectable at baseline by the MDT. 5. Stage IIIC patients. 6. Participants whose planned surgery at enrollment is a wedge resection. 7. Participants contraindicated for surgical intervention due to comorbid conditions. 8. Participants who are allergic to study intervention. 9. Participants with more than one primary tumour. 10. Known active hepatitis infection, positive HCV antibody, HBsAg or HBV core antibody (anti-HBc), at screening. 11. Previously infected or tested positive for human immunodeficiency virus (HIV). 12. Judgement by the investigator that the participant should not participate in the study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: * **Neoadjuvant Period A:** All participants initially receive 2 cycles of durvalumab + CT (investigator's choice platinum-based) every 3 weeks. * **Neoadjuvant Period B:** * *Cohort 1 (Surgery):* Participants deemed eligible by MDT receive 1-2 additional cycles of D+CT,

followed by surgery. * *Cohort 2 (CRT)*: Participants with unresectable tumors per MDT receive definitive CRT (6 one-week cycles) for approximately 6 weeks. * **Adjuvant/Consolidation**: Both cohorts subsequently receive durvalumab every 4 weeks. **Route of Administration**: Intravenous (IV). **Duration of Treatment**: Up to 12 months (1 year) following surgery or completion of CRT, unless disease progression, recurrence, or unacceptable toxicity occurs.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for chemotherapy-induced side effects (e.g., antiemetics). **Prohibited**: Concurrent administration of other investigational anti-cancer therapies or live-attenuated vaccines.

6. Study Timeline & Visit Schedule

Visit ID	Window	Key Activities/Procedures
1	Day -28 to 0	Informed Consent, MDT resectability assessment, FDG-PET, brain MRI, tumor biopsy (EGFR/ALK testing)
2	Weeks 1 to 6	Durvalumab + CT (Cycles 1 & 2 Q3W), Safety Labs
3	Week 6-7	MDT Re-assessment, Re-assessment of resectability by the Multidisciplinary Team
4	Weeks 7 to 15	Surgery / CRT
5	Up to 1 year	Adjuvant Phase: Durvalumab monotherapy every 4 weeks (Q4W), survival & disease progression monitoring

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Resection rate. Resection rate is defined as the proportion of all participants who underwent definitive surgery. Participants who undergo (i.e., start) surgery with the goal of complete tumour resection will be counted as meeting this endpoint. **Measurement Method**: Evaluated post-neoadjuvant treatment via surgical records and Multidisciplinary Team (MDT) operative reports.

7.2 Secondary & Exploratory Endpoints * Resection rates analyzed by baseline resectable vs. borderline-resectable status. * R0, R1, R2 resection rates. * Pathological complete response (pCR) rate. * Event-Free Survival (EFS) and Progression-Free Survival (PFS). * Overall Survival (OS). * Circulating tumor DNA (ctDNA) dynamics pre- and post-surgery correlated with clinical outcomes.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard continuous monitoring for immune-related adverse events (irAEs) and chemotherapy-related toxicities. **Serious Adverse Events (SAEs)**: Standard reporting

timelines per Phase II oncology protocols (typically within 24 hours of site awareness). **Data Monitoring Committee (DMC):** Independent safety monitoring and MDT oversight for dynamic patient allocation between cohorts based on tumor response.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) encompassing all ~142 enrolled patients. **Sample Size Justification:** The target $N=142$ is powered to robustly estimate the overall resection rate and assess pCR across both resectable and borderline-resectable subgroups. **Handling of Missing Data:** Participants who fail to undergo surgery due to disease progression or toxicity will be counted as non-responders for the resection rate primary endpoint.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Rigorous onsite and remote monitoring with dedicated emphasis on the standardized MDT evaluation criteria.

Audit Trail: Comprehensive eCRF logging of MDT baseline and re-assessment decisions to ensure consistent cohort allocation.

11. Study Identifiers & Governance

Primary ID: D9106C00002 **Registry Number:** NCT05925530 **Sponsor/Lead Organization:** AstraZeneca **Study Title:** MDT-BRIDGE **Official Regulatory Title:** A Multicentre, Phase II, Single-Arm, Interventional Study of Neoadjuvant Durvalumab and Platinum-based Chemotherapy (CT), Followed by Either Surgery and Adjuvant Durvalumab or Chemoradiotherapy (CRT) and Consolidation Durvalumab, in Participants with Resectable or Borderline Resectable Stage IIB-IIIB Non-small Cell Lung Cancer (NSCLC)

12. Clinical Framework (NSCLC Specific)

Primary Condition: Non-small Cell Lung Cancer (NSCLC) **Diagnosis Threshold:** Stage IIB to IIIB (N2), EGFR and ALK wild-type **Medical Methodology:** Neoadjuvant Chemoimmunotherapy & Multidisciplinary Team (MDT) routing **Optimization Target:** High Resection Rate and Pathological Complete Response (pCR)

13. Study Procedures & Methodology

Management Pathway: Baseline and post-induction MDT review for surgical eligibility routing **Education Protocol (HCP):** MDT standardized assessment criteria training **Education Protocol (Patient):** Standard oncology informed consent covering potential for surgical vs. CRT pathway **Quality Audit Mechanism:** Tracking of centralized vs. local MDT radiologic and pathologic resectability concordance

14. Observation & Follow-Up Schedule

Total Duration: Up to 1 year post-surgery/CRT (approx. 60+ weeks total including neoadjuvant) **Onsite Visit Cadence:** Every 3 weeks (Neoadjuvant) / Every 4 weeks (Adjuvant/Consolidation) **Remote Monitoring Frequency:** Standard Phase II oncology intervals **Checklist Review Interval:** Restaging scans per standard oncology follow-up guidelines (e.g., every 8-12 weeks)

15. Sign-off & Approval

Role	Name	Signature	Date	:---	:---	:---	:---
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				